PEER REVIEW HISTORY

BMJ Open publishes all reviews undertaken for accepted manuscripts. Reviewers are asked to complete a checklist review form (**http://bmjopen.bmj.com/site/about/resources/checklist.pdf**) and are provided with free text boxes to elaborate on their assessment. These free text comments are reproduced below.

ARTICLE DETAILS

TITLE (PROVISIONAL)	Applicability of predictive models for 30-day unplanned hospital readmission risk in paediatrics: a systematic review
AUTHORS	Niehaus, Ines; Kansy, Nina; Stock, Stephanie; Dötsch, Jörg; Müller, Dirk

VERSION 1 – REVIEW

REVIEWER	Horne, Benjamin Intermountain Medical Center, Intermountain Heart Institute Dr. Horne has developed risk prediction tools for outcomes in adult medicine that are licensed to CareCentra and Alluceo and conducted risk score studies that were funded by AstraZeneca, GlaxoSmithKline, and CareCentra, but has no conflicts of interest to report related to pediatric patients or pediatric healthcare.
REVIEW RETURNED	16-Aug-2021

GENERAL COMMENTS	Niehaus and colleagues reviewed risk prediction models for hospital readmission of pediatric patients. The review was performed using robust methods and the results of the literature search are useful and interesting. The paper is written well and contributes to the generalizable knowledge in the field. The paper would be improved with the following changes: Major Comments: 1. In the Introduction, second paragraph, second sentence, it says, "...identification of patients with an increased risk of HR..." I think that the authors mean "higher" or "greater" risk since the risk models are being evaluated at one time point and not changes in the risk over multiple time points. It appears that the risk models in the paper do not evaluate changes in parameters over time but give a snapshot at a specific time point, so there is no increase that is observed but just a higher or greater risk at a specific timepoint. 2. The final paragraph of the Introduction is confusing. It says that there are two research gaps, but it isn't clear in the text leading up to that what those gaps may be and they are not clearly defined in the first sentence of the final paragraph. Further, the second sentence then simply indicates that the paper provides an overview of the existing models and the third sentence discusses the objective of the review that would be achieved through three components of the evaluation. A re-write of this paragraph would help clarify the intentions of the authors, the objective of the review, and what research gaps exist. 3. In the Discussion or Conclusion, it would be helpful to mention examples of major gaps in knowledge that the review identified or specific areas for predictive modeling development that are currently lacking for pediatric hospital readmission. It appears that the models

	that do exist do not necessarily compete with each other, so considerable room for additional risk modeling exists. 4. Also, in the Discussion or Conclusion, it would be interesting to mention if some of the models are only moderately able to predict outcomes and better models need to be developed in a specific area? 5. Finally, what about the usability of the models? Are some more practical for implementation in clinical practice, are they all usable, or are none very practical for actual use although they predict readmission?

REVIEWER	Anderson, Candice James Cook University College of Medicine and Dentistry
REVIEW RETURNED	11-Sep-2021

GENERAL COMMENTS	Data source and source strategy: - Stated "according to previous systematic reviews, the key search terms included" - this is an ambitious statement and does not highlight your search terms. I would re-write the statement to place emphasis on what you used. Applicability and significant risks: - Did the papers describe what constituted as illness severity - in the context on acute vs chronic medical conditions? Similarly what constituted as health service use prior to admission? If so, I would recommend that you elaborate on these factors.

REVIEWER	Zhou, Huaqiong Curtin University, School of Nursing, Midwifery and Paramedicine
REVIEW RETURNED	20-Sep-2021

GENERAL COMMENTS	Thank you for the opportunity to review this well-structured manuscript! The manuscript is, however, not recommended for publication at this stage as the authors need to address following questions on the selection of studies for the systematic review: 1. Based on the “Inclusion criteria” on Page 6 of 45/PDF, “No original article (n = 37); Study population not in scope (n = 35); No 30-day HR outcome (n = 78)” (Refers to page 8 and Figure 1/PRISMA flow chart on Page 26) should be excluded from the stage of “Records screened” based on the title and abstract. 2. Authors need to clarify what means “No study for individualized risk prediction (n = 45)”. I question if these 45 records are also should be excluded at the stage of “Records screened”. 3. Authors need to elaborate on the 20 excluded studies with mixed outcome. 4. Authors stated on Page 8 of 45 under “Search result” that “Of the 57 excluded prognostic factor studies for 30-day paediatric HRs, nine studies provided information about discriminative ability, where the c-statistic ranged from 0.61 to 0.79[35-43].” I have gone through the nine studies and noted these 9 studies met the inclusion criteria on Page 6 of 45/PDF but authors excluded them. Authors need to explicitly justify the exclusion criteria on “prognostic factor studies”. 5. Further articles to be considered for inclusion in the systematic review are as following:

	• Taylor et al. (2020). Development and validation of a web-based pediatric readmission risk assessment tool. Hospital Pediatrics. http://doi.org/10.1542/hpeds.2019-0241 • Brown et al. (2019). Biomarkers improve prediction of 30-day unplanned readmission or mortality after paediatric congenital heart surgery. Cardiology in the Young. http://doi.org/10.1017/S1047951119001471 • Janjua et al. (2019). Predictors of 90-day readmission in children undergoing spinal cord tumor surgery: A nationwide readmissions database analysis. World Neurosurg. http://doi.org/10.1016/j.wneu.2019.03.245 • Vedantam et al. (2018). Thirty-day outcomes in pediatric epilepsy surgery. Childs Nervous System. http://doi.org/10.1007/s00381-017-3639-z • Just a suggestion: considering the timeframe (almost end of September 2021), if the authors are willing to extend the search period/update the search, our research team has also published predictive models development using machine learning in April 2021. The manuscript was accepted in June 2020 and reference is as below: Zhou et al. (2021). Using machine learning to predict paediatric 30-day unplanned hospital readmissions: A case-control retrospective analysis of medical records, including written discharge documentation. Australian Health Review. https://doi.org/10.1071/AH20062 Minor comments for authors' considerations as below: • Table 1, Reference Ehwerhemuepha et al. 2020: please add "unplanned" to 30-day HRs. Please also check Smith et al. 2015 and Jiang et al. 2018; • If all studies examined 30-day unplanned HRs, Author could consider removing the column of "Model outcome"; Authors should also consider adding "unplanned" in the title of this manuscript • Table 1 on Page 11 of 45 PDF, Authors need to closely check spelling on "Dez." (3 times in the table) – did you mean "Dec."? • Table 1 "Period of data collection", if it Jan. – Dec. 2014 means entire Year of 2014, then please just state 2014 OR "Jan. 2010 – Dec. 2014" as 2010-2014. There are multiple cells to be checked.

VERSION 1 – AUTHOR RESPONSE

Reviewer: 1
Dr. Benjamin Horne, Intermountain Medical Center, Stanford University

Comments to the Author:
Niehaus and colleagues reviewed risk prediction models for hospital readmission of pediatric patients. The review was performed using robust methods and the results of the literature search are useful and interesting. The paper is written well and contributes to the generalizable knowledge in the field.

The paper would be improved with the following changes:

Major Comments:
1. In the Introduction, second paragraph, second sentence, it says, "...identification of patients with an increased risk of HR..." I think that the authors mean "higher" or "greater" risk since the risk models are being evaluated at one time point and not changes in the risk over multiple time points. It appears that the risk models in the paper do not evaluate changes in parameters over time but give a snapshot at a specific time point, so there is no increase that is observed but just a higher or greater risk at a specific timepoint.

Response (point 1): We have rewritten the sentence.

2. The final paragraph of the Introduction is confusing. It says that there are two research gaps, but it isn't clear in the text leading up to that what those gaps may be and they are not clearly defined in the first sentence of the final paragraph. Further, the second sentence then simply indicates that the paper provides an overview of the existing models and the third sentence discusses the objective of the review that would be achieved through three components of the evaluation. A re-write of this paragraph would help clarify the intentions of the authors, the objective of the review, and what research gaps exist.

Response (point 2): Thank you for your comment. We have rewritten the last and penultimate paragraphs from the introduction and hope that to have clarified the section.

3. In the Discussion or Conclusion, it would be helpful to mention examples of major gaps in knowledge that the review identified or specific areas for predictive modeling development that are currently lacking for pediatric hospital readmission. It appears that the models that do exist do not necessarily compete with each other, so considerable room for additional risk modeling exists.
4. Also, in the Discussion or Conclusion, it would be interesting to mention if some of the models are only moderately able to predict outcomes and better models need to be developed in a specific area?

Response (point 3&4): We have revised the second paragraph for discussion of future research.

5. Finally, what about the usability of the models? Are some more practical for implementation in clinical practice, are they all usable, or are none very practical for actual use although they predict readmission?

Response (point 5): We have added a new paragraph in the discussion section ‘Practical clinical and policy implications’ to discuss the usability of predictive models in practice.

Reviewer: 2
Miss Candice Anderson, James Cook University College of Medicine and Dentistry

Comments to the Author:
Data source and source strategy:
- Stated "according to previous systematic reviews, the key search terms included" - this is an ambitious statement and does not highlight your search terms. I would re-write the statement to place emphasis on what you used.

Response: We have rewritten the relevant section to make our focus on search terms clear.

Applicability and significant risks:
- Did the papers describe what constituted as illness severity - in the context on acute vs chronic

medical conditions? Similarly what constituted as health service use prior to admission? If so, I would recommend that you elaborate on these factors.

Response: For the two risk factor categories, we have added explanatory footnotes in Table 2 (see footnote e&f). We have also re-listed relevant examples in the section 'Applicability and significant risk factors in predictive models'. Unfortunately, it is not possible to distinguish between acute and chronic medical conditions based on the identified significant risk factors regarding illness severity.

Reviewer: 3
Ms. Huaqiong Zhou, Curtin University

Comments to the Author:
Thank you for the opportunity to review this well-structured manuscript! The manuscript is, however, not recommended for publication at this stage as the authors need to address following questions on the selection of studies for the systematic review:

1. Based on the "Inclusion criteria" on Page 6 of 45/PDF, "No original article (n = 37); Study population not in scope (n = 35); No 30-day HR outcome (n = 78)" (Refers to page 8 and Figure 1/PRISMA flow chart on Page 26) should be excluded from the stage of "Records screened" based on the title and abstract.

Response: We have excluded the records from the stage "Reports sought for retrieval" (see figure 1).

2. Authors need to clarify what means "No study for individualized risk prediction (n = 45)". I question if these 45 records are also should be excluded at the stage of "Records screened".

Response: We have excluded the records from the stage "Reports sought for retrieval" (see figure 1).

3. Authors need to elaborate on the 20 excluded studies with mixed outcome.

Response: We have elaborated on the excluded studies with mixed outcome in the result chapter (section: search results). Please consider that instead of 20 excluded studies with mixed outcome, there are 12 studies left as the other studies had next to "mixed outcome" "No 30-day HR outcome" as an exclusion criteria. We have excluded "No 30-day HR outcome" from this stage of search (see point 1). Studies that have fulfilled more than 2 exclusion criteria were not captured twice in figure 1 (original version).With your recommendation to exclude some records from the stage "Reports sought for retrieval" (see point 1&2), records do not fulfill 2 or more exclusion criteria (except Janjua et al. 2019 captured as predictive model without discrimination, at the same time machine learning model clarified in table A7).

4. Authors stated on Page 8 of 45 under "Search result" that "Of the 57 excluded prognostic factor studies for 30-day paediatric HRs, nine studies provided information about discriminative ability, where the c-statistic ranged from 0.61 to 0.79[35-43]." I have gone through the nine studies and noted these 9 studies met the inclusion criteria on Page 6 of 45/PDF but authors excluded them. Authors need to explicitly justify the exclusion criteria on "prognostic factor studies".

Response: We have revised the second paragraph on our inclusion criteria and added prognostic factor studies with discrimination in table A7.

5. Further articles to be considered for inclusion in the systematic review are as following:

• Taylor et al. (2020). Development and validation of a web-based pediatric readmission risk assessment tool. Hospital Pediatrics. http://doi.org/10.1542/hpeds.2019-0241

Response: As part of our systematic review, we excluded machine learning based predictive models from the analysis. Because this has not become entirely clear, we have revised the second paragraph to our inclusion criteria. In addition, we have included the suggested study in the discussion and presented it separately in Table A7 (thanks for the recommendation).

• Brown et al. (2019). Biomarkers improve prediction of 30-day unplanned readmission or mortality after paediatric congenital heart surgery. Cardiology in the Young. http://doi.org/10.1017/S1047951119001471

Response: Studies with a mixed outcome were excluded from the analysis. We have listed the study as an excluded study under the search results section.

• Janjua et al. (2019). Predictors of 90-day readmission in children undergoing spinal cord tumor surgery: A nationwide readmissions database analysis. World Neurosurg. http://doi.org/10.1016/j.wneu.2019.03.245

Response: Studies with no discrimination metric were excluded from the analysis. We listed the study under the search results section as an excluded study and presented it separately in Table A7.

• Vedantam et al. (2018). Thirty-day outcomes in pediatric epilepsy surgery. Childs Nervous System. http://doi.org/10.1007/s00381-017-3639-z

Response: Thank you for pointing out this study, which we did not capture in our electronic database query. We have classified the study as a “prognostic factor study with discrimination”, added it to Figure 1, listed it as an excluded study under the search results section, and presented it separately in Table A7.

• Just a suggestion: considering the timeframe (almost end of September 2021), if the authors are willing to extend the search period/update the search, our research team has also published predictive models development using machine learning in April 2021. The manuscript was accepted in June 2020 and reference is as below:

Zhou et al. (2021). Using machine learning to predict paediatric 30-day unplanned hospital readmissions: A case-control retrospective analysis of medical records, including written discharge documentation. Australian Health Review. https://doi.org/10.1071/AH20062

Response: We have extended our search period (7th October). The regression-based predictive models are included in the analysis. The machine learning predictive models are mentioned in the discussion and presented separately in Table A7.

Minor comments for authors’ considerations as below:
• Table 1, Reference Ehwerhemuepha et al. 2020: please add “unplanned” to 30-day HRs. Please also check Smith et al. 2015 and Jiang et al. 2018;

Response: We have adjusted the outcome definition. The outcomes for the other studies have remained the same.

• If all studies examined 30-day unplanned HRs, Author could consider removing the column of “Model outcome”; Authors should also consider adding “unplanned” in the title of this manuscript

Response: Due to our comment above this is not applicable.

• Table 1 on Page 11 of 45 PDF, Authors need to closely check spelling on “Dez.” (3 times in the table) – did you mean “Dec.”?

Response: We have corrected the spelling.

• Table 1 “Period of data collection”, if it Jan. – Dec. 2014 means entire Year of 2014, then please just state 2014 OR “Jan. 2010 – Dec. 2014” as 2010-2014. There are multiple cells to be checked.

Response: We have standardized the time specifications.

VERSION 2 – REVIEW

REVIEWER	Horne, Benjamin Intermountain Medical Center, Intermountain Heart Institute Dr. Horne is an inventor of clinical decision tools that are licensed to CareCentra and Alluceo, and a member of the scientific advisory board of LabMe. He is also the PI of grants involving clinical decision tools that were funded by the Intermountain Research and Medical Foundation, CareCentra, GlaxoSmithKline, and AstraZeneca, and is a co-investigator of a grant funded by the Patient-Centered Outcomes Research Institute (PCORI).
REVIEW RETURNED	01-Nov-2021

GENERAL COMMENTS	The authors have responded thoroughly to the prior review comments and modified the manuscript accordingly. I have no further comments on the paper.

REVIEWER	Anderson, Candice James Cook University College of Medicine and Dentistry
REVIEW RETURNED	22-Nov-2021

GENERAL COMMENTS	Good review - thank you for considering the recommended changes.

REVIEWER	Zhou, Huaqiong Curtin University, School of Nursing, Midwifery and Paramedicine
REVIEW RETURNED	24-Nov-2021

GENERAL COMMENTS	Thank you for the opportunity to review the revised manuscript. Noted and appreciated the changes that authors had made to extend the database search to October 2021 and included three more studies in the review. However, two major comments remained undressed or not addressed clearly are listed as below 1. The justification to exclude “prognostic studies” (n=11) remains unclear. Of the 11 studies, Reference 50 should be excluded at an earlier stage as the outcome measures was 90-day readmission. However the rest of 10 studies (Reference 31, 52-60) fulfil your inclusion criteria of (1) predicting 30-day readmission following the index admission of either a surgery or medical admission; (2)

	multivariable regression analysis; and (3) reporting of risk predictive model performance of c-statistic. References 31, 52-55, 57-60 should be grouped under the surgical related unplanned hospital readmissions and Reference 56 should be grouped under the medical related readmission. For example, Reference 31 (Vedantam et al., 2018) performed multivariable logistic regression analysis using perioperative data to identify risk factors for adverse events within 30 days of the index procedure. The study identified that patients undergoing hemispherectomy (OR=4.11, 95% CI 1.48–11.42, p = 0.01) was significantly associated with higher risk of unplanned readmission after pediatric epilepsy surgery. The study design of Reference 31 is no different from one of your included 16 studies (Reference 73, Smith et al., 2015). Authors need to closely examine the 10 studies (References 31, 52-60) to avoid omission of valuable research evidence. 2. Majority of the included studies in the review examined unplanned hospital readmission. However this was not reflected in the title or aim. Minor comments Consistency and accuracy presentation of the RISMA flow chart • 2nd text box on the left column: Records screened – was the title and abstract were screened? • Inconsistent use of terms – “records” vs. “Reports” • Last text box on the right column: Hand search – was it meant to be “16” instead of “13”

VERSION 2 – AUTHOR RESPONSE

Reviewer: 1
Dr. Benjamin Horne, Intermountain Medical Center, Stanford University

Comments to the Author:
The authors have responded thoroughly to the prior review comments and modified the manuscript accordingly. I have no further comments on the paper.

Response: Thank you for reviewing the revised manuscript. Please note that we have expanded the systematic review by 14 studies based on one reviewer comment. We have, of course, maintained your comments from the first revision.

Reviewer: 2
Miss Candice Anderson, James Cook University College of Medicine and Dentistry

Comments to the Author:
Good review - thank you for considering the recommended changes.

Response: Thank you for reviewing the revised manuscript. Please note that we have expanded the review by 14 studies based on one reviewer comment. We have, of course, maintained your comments from the first revision.

Reviewer: 3
Ms. Huaqiong Zhou, Curtin University

Comments to the Author:
Thank you for the opportunity to review the revised manuscript. Noted and appreciated the changes that authors had made to extend the database search to October 2021 and included three more studies in the review. However, two major comments remained undressed or not addressed clearly are listed as below
1. The justification to exclude “prognostic studies” (n=11) remains unclear.

Of the 11 studies, Reference 50 should be excluded at an earlier stage as the outcome measures was 90-day readmission.

However the rest of 10 studies (Reference 31, 52-60) fulfil your inclusion criteria of (1) predicting 30-day readmission following the index admission of either a surgery or medical admission; (2) multivariable regression analysis; and (3) reporting of risk predictive model performance of c-statistic.

References 31, 52-55, 57-60 should be grouped under the surgical related unplanned hospital readmissions and Reference 56 should be grouped under the medical related readmission.

For example, Reference 31 (Vedantam et al., 2018) performed multivariable logistic regression analysis using perioperative data to identify risk factors for adverse events within 30 days of the index procedure. The study identified that patients undergoing hemispherectomy (OR=4.11, 95% CI 1.48–11.42, p = 0.01) was significantly associated with higher risk of unplanned readmission after pediatric epilepsy surgery. The study design of Reference 31 is no different from one of your included 16 studies (Reference 73, Smith et al., 2015).

Authors need to closely examine the 10 studies (References 31, 52-60) to avoid omission of valuable research evidence.

Response: Based on your comment we have decided to include the 11 studies in the review. Please note that we also include the study of Chotai et al. 2017 as next to a 90-day HRs predictive model a 30-day HRs predictive model was developed. Three further studies were included by the hand search of reference lists and based on your recommendation concerning the inclusion criteria.
Please note that we have excluded the 11 studies before as they aimed to identify 30-day HR predictors. Therefore, we have classified those studies as prognostic factor studies and thus excluded from the analysis before (so as not to bias them adversely in TRIPOD adherence).

2. Majority of the included studies in the review examined unplanned hospital readmission. However this was not reflected in the title or aim.
Response: Thank you for that comment. We have revised the last part of the introduction and the result part oft the abstract. We also have clarified our inclusion criteria (cf. Method chapter). Furthermore, we have changed the title to clarify our outcome selection. In addition, we discuss our outcome selection in the context of the chapter ‘Limitations’.

Minor comments
Consistency and accuracy presentation of the RISMA flow chart
• 2nd text box on the left column: Records screened – was the title and abstract were screened?
• Inconsistent use of terms – “records” vs. “Reports”
• Last text box on the right column: Hand search – was it meant to be “16” instead of “13”

Response: We have revised the PRISMA flow chart. Please take into account that the PRISMA flow

chart is now placed in our supplemental material (see figure A1) as according to the editorial office tables are not allowed to exceed 2 pages. Therefore we have to split table 1 into one table for all-cause HRs and into a second table for surgical and medical conditions related HRs. The core steps of our study selection you can find in the chapter 'search result'.

VERSION 3 – REVIEW

REVIEWER	Zhou, Huaqiong Curtin University, School of Nursing, Midwifery and Paramedicine
REVIEW RETURNED	13-Jan-2022

GENERAL COMMENTS	Thank you for the opportunity to review the 2nd revision. The addition of 14 studies in this systematic review is greatly appreciated. However, three main comments to be fully addressed are listed as below 1. Authors have deleted the justification of exclusion "prognostic factor studies" on the PDF Proof Page 6 Line 12, however there were 52 prognostic factor studies. Authors need to provide a clear definition of prognostic factor studies. 2. Authors stated, of the 30 included studies, 28 focused on unplanned hospital readmissions on PDF Proof Page 8 Line 57. Two studies within Table 2 were indicated as hospital readmissions, which measured both unplanned and planned readmissions. I appreciate that authors justified under the Limitations on PDF Page 18/19 that "(iii) With regard to the inclusion criteria, we defined the outcome as 30-day HRs for predictive models that can address both planned and unplanned HRs. The resulting heterogeneity was accepted because in an earlier systematic review a wider definition of HRs had proved useful[8]". However, since 28 studies were only unplanned hospital readmissions it would only be reasonable to exclude the two studies. Authors should then revise the manuscript to unplanned hospital readmission throughout the manuscript. 3. Discussion was mainly based on predictive model performance but there is not discussion of the three most commonly cited predictors – which might be more relevant to clinical practice rather than c-statistics for clinicians. The conclusion on Page 20 Line 24-27 " To increase accessibility for use, more attention should be given on completeness in reporting, particularly for items that may be relevant for the implementation of paediatric 30-day HR predictive models in practice" is rather general. Authors should be more specific on "items that may be relevant…"

VERSION 3 – AUTHOR RESPONSE

Reviewer: 3
Ms. Huaqiong Zhou, Curtin University
Comments to the Author:
Thank you for the opportunity to review the 2nd revision. The addition of 14 studies in this systematic review is greatly appreciated. However, three main comments to be fully addressed are listed as below

1. Authors have deleted the justification of exclusion “prognostic factor studies” on the PDF Proof Page 6 Line 12, however there were 52 prognostic factor studies. Authors need to provide a clear definition of prognostic factor studies.

Response: We have included the definition for prognostic factor studies within the ‘inclusion criteria’ section.

2. Authors stated, of the 30 included studies, 28 focused on unplanned hospital readmissions on PDF Proof Page 8 Line 57. Two studies within Table 2 were indicated as hospital readmissions, which measured both unplanned and planned readmissions. I appreciate that authors justified under the Limitations on PDF Page 18/19 that “(iii) With regard to the inclusion criteria, we defined the outcome as 30-day HRs for predictive models that can address both planned and unplanned HRs. The resulting heterogeneity was accepted because in an earlier systematic review a wider definition of HRs had proved useful[8]”. However, since 28 studies were only unplanned hospital readmissions it would only be reasonable to exclude the two studies. Authors should then revise the manuscript to unplanned hospital readmission throughout the manuscript.

Response: We have removed the two studies from the analysis and noted the change in the first paragraph of the ‘method’ section. Because an unpublished study protocol exist, the deviation from this protocol was mentioned.

3. Discussion was mainly based on predictive model performance but there is not discussion of the three most commonly cited predictors – which might be more relevant to clinical practice rather than c-statistics for clinicians. The conclusion on Page 20 Line 24-27 “ To increase accessibility for use, more attention should be given on completeness in reporting, particularly for items that may be relevant for the implementation of paediatric 30-day HR predictive models in practice” is rather general. Authors should be more specific on “items that may be relevant…”

Response:
We added a paragraph in the discussion about the relevance of different risk factors (see last paragraph of the section ‘Practical clinical and policy implications’).

In addition, we have included a more detailed description of relevant items in the last sentence of the conclusion.

VERSION 4 – REVIEW

REVIEWER	Zhou, Huaqiong Curtin University, School of Nursing, Midwifery and Paramedicine
REVIEW RETURNED	07-Feb-2022

GENERAL COMMENTS	Thank you for addressing all the comments! Time for publication!